SAMPLE ANSWER · STROKE · CASE 5

Sample 7-minute ISBAR: Hannah

Hannah Kowalski · 29-year-old woman · Community rehabilitation service, outpatient appointment · about 1046 spoken words (≈ 7.5 min at 140 words per minute)

I Introduction

62 words

"Hi, I'm the occupational therapy student who has been seeing Hannah Kowalski today. Hannah is 29 and had a left posterior cerebral artery, or occipital, stroke eight weeks ago, two weeks after giving birth. I need your help urgently because she's about to drive home with her 10-week-old baby, and I've seen signs that she may be missing things on her right."

S Situation

124 words

"Hannah came in to plan her return to teaching. During the session she left out words at the right-hand end of lines when reading, didn't find a page I placed on her right until I pointed to it, and startled when a colleague walked in on her right.

She described bruising her right hip twice on the kitchen bench, a near miss with a car reversing on her right in a car park, catching Mia's capsule on the stair rail, and twice not noticing the hot tap running while bathing Mia.

She has been driving for the past few weeks, drove here today with Mia and plans to stop at the shops. Mia is crying, and Hannah has picked up her keys to leave."

B Background

142 words

"Hannah's stroke was eight weeks ago, two weeks postpartum, and her daughter Mia is now 10 weeks old. She walks independently and has had no falls. Last week she scored 28 out of 30 on a cognitive screen. She says her GP recently told her she's fine. She's a primary school teacher on parental leave and lives in a two-storey townhouse with her partner Ryan, who works fly-in fly-out and is away for two weeks. Her mother in New Zealand has offered to come and help, but Hannah doesn't want to 'make a fuss'.

There are important gaps. I couldn't find any documentation that her vision has been formally assessed since the stroke, or that she has had driving advice. I don't know exactly what the GP reviewed, and I don't know what support she has at home while Ryan is away."

A Assessment

373 words

"My main concern is that Hannah's pattern may indicate a loss of vision on the right side of both eyes. That would fit a left occipital stroke, because that part of the brain processes vision from the opposite side. I can't diagnose it, but the signs are consistent: reading, objects on the desk, people approaching, the bench, the car and the tap all involve her right side. She appears to compensate partly by turning her head, but she has had several near misses.

The most immediate risk is driving. A visual field loss can mean she doesn't see a car, cyclist or child coming from the right, and Mia is in the car with her. Austroads' Assessing Fitness to Drive has standards for both stroke and visual fields, and someone with this pattern needs formal vision testing and medical advice before driving. In Queensland, licence holders must

also tell the Department of Transport and Main Roads about a medical condition that could affect their driving.

The second risk is Mia's care at home. Twice not noticing the hot tap creates a scald risk, and carrying Mia on the stairs while catching the capsule on the rail creates a fall risk. Hannah is doing this alone for the next two weeks.

I also want to look past the reassuring parts. A score of 28 out of 30 on a cognitive screen doesn't test visual fields, so it can't rule this out. Walking independently and having no falls don't show how she copes with traffic or a crowded playground. And her GP saying she's 'fine' may not have included a field test; we don't know. Hannah is articulate and confident, which makes it easy to miss.

Her return-to-teaching plan is a real and important goal, and this visual change is directly relevant to it, for reading and for supervising children. But it isn't the priority today.

So, immediately, the priority is that she doesn't drive today and that she gets home safely with Mia. Next is formal vision assessment and medical driving advice, then infant-care safety at home, and later, return to work. I can't stop her driving or decide her fitness to drive, but I think we must clearly advise her and escalate today."

R Recommendation

345 words

"First, I'd ask Hannah to stay a few more minutes and offer her a quiet room to feed Mia. That settles Mia and gives us time. Then I'd explain what I noticed, kindly and specifically, and advise her not to drive today until she has had medical advice. I'd like you to join me for that conversation. I'd help her find another way home with Mia safely restrained, for example a friend or neighbour collecting them, and offer to call someone for her.

Second, I'd like your help to contact her GP or the rehabilitation medical team today, so she can be reviewed for formal visual field testing, such as perimetry through an optometrist or ophthalmology, and receive clear driving advice in line with Austroads. If she chooses to drive anyway, I'd follow our service procedure with you and document the advice given.

Third, the MDT. I'd ask social work to help with support while Ryan is away, including whether her mother's offer to come over could work. I'd suggest a referral to the local child health nurse, who could support Mia's care and check on Hannah's wellbeing, given a new baby, a stroke and her partner away.

For OT, the principal short-term assessment I'd recommend is a home visit to observe her performance of Mia's bath and carrying her on the stairs. That shows the real risks in her own environment. Simple strategies could follow straight away: running the bath and checking the temperature before bringing Mia in, keeping the taps on her left side where possible, and setting up a change station downstairs.

For the longer term, I'd use the Canadian Occupational Performance Measure, so Hannah's own priorities, like caring for Mia, getting back to teaching and driving again, guide her rehabilitation. The Stroke Foundation Living Clinical Guidelines support visual scanning training for visual field loss, which could help with reading, community mobility and eventually work. If she's medically cleared later, an OT driver assessment could follow.

Finally, I'd book a follow-up this week, including the letter to her principal she asked for."

Coaching notes *not spoken*

The immediate priority

Hannah's signs consistently suggest she is missing things on her right, and she is about to drive home with her baby. Advise her not to drive, arrange a safe way home, and escalate today for formal vision testing and medical driving advice. Infant-care safety at home comes next.

Distractors in this case

- **28/30 cognitive screen** — doesn't assess visual fields.
- **Walks independently, no falls** — mobility on foot isn't the same as scanning traffic.
- **"My GP said I'm fine"** — we don't know what was assessed.
- **Articulate and confident; return-to-teaching letter** — a valid goal, but not today's priority.
- **Crying baby and car keys** — real time pressure; offer a quiet room to feed rather than letting it end the session.

Missing information to clarify

- Whether her vision has been formally assessed since the stroke
- Whether she has had any driving advice
- What the GP reviewed
- Practical support while Ryan is away; whether her mother can come

Useful references

- Austroads, *Assessing Fitness to Drive* (vision and visual fields; stroke)
- Queensland Department of Transport and Main Roads — medical conditions and driver licensing
- Stroke Foundation, *Living Clinical Guidelines for Stroke Management* (visual field loss)

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